



DR. B. S. AJAIKUMAR

FOUNDER, HEALTHCARE GLOBAL ENTERPRISES

(FY26 REVENUE: ₹2,545 CR)

After training in oncology in the United States, Dr B.S. Ajaikumar returned to India in 2003 to establish Healthcare Global Enterprises (HCG), with the goal of building a specialised cancer-care network that extended advanced oncology services beyond India's major metropolitan cities. HCG adopted a hub-and-spoke model, anchoring specialised cancer centres in larger cities while linking them to smaller facilities in Tier-II and Tier-III towns, and invested significantly in radiation therapy, precision medicine and molecular diagnostics to keep pace with evolving cancer care standards.

ment, given how quickly the rupee has been depreciating against other currencies, and has pushed the group into partnerships with Indian manufacturers to build equipment domestically.

The group runs 42 facilities today with just over 5,000 operational beds, and is looking at further expansion in the Caribbean, where it already has a presence. That expansion has already reached further than the Caribbean: in November 2025, Narayana completed the acquisition of UK-based Practice Plus Group Hospitals, and by the year ended March 2026, its group revenue had risen 44% to nearly ₹7,900 crore, with the UK business contributing for the first time. As Dr. Shetty puts it, the work so far is “nothing compared to what we can and should do.”

DIFFERENT VISIONS, ONE GOAL

If Dr. Prathap C. Reddy laid the foundations of India's corporate hospital sector, those who followed built on them in very different ways. They shared an entrepreneurial instinct, but not the same vision. Some focused on making complex healthcare affordable. Others set out to build integrated institutions, expand organised healthcare into underserved regions, or consolidate hospital networks through acquisitions. Different paths, but a common purpose: making quality healthcare more accessible to millions of Indians.

Dr. Naresh Trehan, Chairman and Managing Director of Medanta, returned to India after spending nearly 20 years in the United States with a clear vision of the kind of institution he wanted to build. “My focus was not simply on adding hospital capacity, but on creating an integrated healthcare institution that combined advanced clinical care, research, medical education and multiple specialties under one roof,” he says.

Even as India's hospital sector expands rapidly, Dr. Trehan believes the country's biggest challenge lies elsewhere. The healthcare workforce, he says, remains one of India's most pressing concerns.

Dr. Dharminder Nagar, Managing Director of Paras Health, picked a gap that none of the above were particularly interested in filling: North India outside the metros. “The market in North India was severely underserved,” he says, pointing out that the region accounts for roughly 60% of the country's population but a much smaller share of its organised healthcare capacity.

When his first hospital opened in Gurugram at the end of 2006 with 200 beds, the largest existing hospital in the city had 30. He went on to build in Patna, then Darbhanga, then Srinagar and Kanpur, cities most hospital chains still consider too small to be worth the effort.

His argument, repeated in nearly every conversation he has on the subject, is that “healthcare has to be delivered where you live,” and that the assumption smaller